



Health Claims Forum

Top Tips for Misrepresentation

Make sure the misrepresentation is ‘qualifying’

i.e terms would actually have been affected (i.e. a rating, exclusion, postponement or decline)

Was there an appropriately worded question under which we could reasonably have expected disclosure?

Don’t just assume there to be. This needs to be well documented in your write up and retro UW referral

Always consider the wording of questions.

Overly long or catch-all questions do not encourage accurate disclosure.

Was someone actually diagnosed with a ‘disorder’?

Is it reasonable to expect disclosure of something that happened 4.5 years ago against a 5-year question?

How significant was the event? Someone is much more likely to forget an isolated raised BP reading as opposed to a suicide attempt

What was the customer advised at the time? Were they copied in to relevant letters?

Keep in mind that some misrepresentation will be reported only with the benefit of hindsight once the customer has received a critical diagnosis. Very little significance may have been placed on the history at the time it was actually taking place or at the point of application

Top Tips for TCF Calls

Don't fear TCF calls, think of them as you would any other call with a customer such as an update call
Explain to the customer that you've noted some discrepancies in the medical evidence received versus what was advised at application and you need them to help you understand why they might not have advised of this history at the point of application when asked a relevant question
Prepare for your call Make sure you know your case, the key facts, specific dates and what was discussed with the customer at the time. Having a summary and / or a timeline of the misrep. in front of you is useful
Listen to the customer! Although you will have in mind the points you wish to cover off in your call, remember that you need to react and amend your conversation in response to the information the customer is providing. Don't be afraid to tell the customer that you need to go away and relook at the evidence in light of what they've advised (or suggest that they do this), and that you'll call them back again at a suitable time to reconvene
Don't be afraid to probe or question the customer more. You should not come away from the call unless you feel that you really understand why the customer didn't advise of their relevant history

If the customer contradicts what you're seeing in the medical evidence you should question them further on this Discuss what you have in black and white in front of you If the customer's explanation doesn't make sense or stack up, tell them you still don't understand and probe further	Don't lead the customer – rely on open questions. For example, when discussing smoking it is best to simply ask the customer to tell you about their smoking habits over the years rather than simply asking about the 12 months pre-app Pick your battles but also don't ever assume! Some cases of misrepresentation will be extremely clear and / or the customer will be wholly reluctant to divulge any information. TCF calls on such cases may therefore be relatively short. The facts will speak for themselves and the customer hasn't or won't be able to provide any reasonable explanation as to why the matter/s in hand weren't advised. However never assume that the customer won't have a feasible explanation – there can always be a reason why!	Silence is golden Although you will have to lead the call and set the scene for the customer, don't feel the need to fill all silences and do the majority of the talking. Often allowing a silence to go on for a little longer than typically comfortable will elicit a further response from the customer
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Additional Explanatory notes from the ABI Code of Practice

Category	Description	Notes from ABI Guide
Innocent	- The customer has acted honestly and reasonably in all of the circumstances, including the customer's individual circumstances but only where these were known to the insurer - In the circumstances, a reasonable person would have considered that the information was relevant to the insurer - The misrepresentation would have resulted in a different underwriting outcome	Section 7 ABI Code of Practice 7.1 Typical characteristics: 7.1.1 The question was not clear enough – any ambiguous wording should be construed in favour of the customer. 7.1.2 The question did not apply clearly to the facts in question 7.1.3 It was reasonable for the customer to have overlooked omitted information – for example, a minor childhood ailment. ***** 7.2 It is irrelevant whether or not there is a link between the misrepresentation and the cause of the claim.
Careless	Applies where the misrepresentation resulted from insufficient care - the failure to exercise reasonable care. this includes anything from an understandable oversight or an inadvertent mistake to serious negligence	Section 9 ABI Code of Practice 9.1 Includes all cases between innocent and deliberate or reckless

Category	Description	Notes from ABI Guide
	- In the circumstances, a reasonable person would have considered that the information was relevant to the insurer - The misrepresentation would have resulted in a different underwriting outcome	
Deliberate/Reckless	- Only applies where the misrepresentation was deliberate or reckless - In the circumstances, on the balance of probabilities, the customer knew, or must have known that the information given was both incorrect and relevant to the insurer or the customer acted without any care as to whether it was either correct or relevant to the insurer	Section 8 ABI Code of Practice 8.1 The overall principle is that the severe remedy of avoiding a policy from outset should be confined to the most serious cases of misrepresentation 8.2 The insurer has the initial burden of establishing whether any case falls into this category on the balance of probabilities. Accordingly, insurers need clear evidence to show this applies ***** 8.3 This category does not apply where: 8.3.1 Having investigated the matter, the customer has a credible explanation supported by the facts for the misrepresentation having occurred and/or there are other credible mitigating circumstances 8.3.2 The degree of relevance associated with the misrepresentation is relatively low and, in cases where a premium rating would have applied, the underlying risk premium rating resulting from that misrepresentation would not have been more than +50% (or £1/mil) for the applicable life assured. ***** 8.4 Typical characteristics: 8.4.1 Deliberate – in the circumstances, the customer knew, or must have known, that the representation they made in answer to a question was incorrect and knew, or must have known, that the information was relevant to the insurer.

Category	Description	Notes from ABI Guide
		8.4.2 Reckless – it is clear that the customer had a complete disregard for the question or the accuracy of the answer when completing the application and must have understood that the information was relevant to the insurer. ***** 8.4.3 Medical information – in cases where the misrepresentation was about the customer’s medical history or family medical history (as opposed to, for example, about occupation, time spent abroad, the use of tobacco products, alcohol or drugs) insurers should take into account that in some circumstances consumers may not have a full understanding of their medical history. Accordingly, this category is more likely to apply in the following situations where: • 8.4.3.1 misrepresented information is widely known by consumers to be important to the risk of a claim being made (for example, cancer, heart disease, diabetes or, for income protection, periods of time off work as a result of incapacity) • 8.4.3.2 misrepresented information concerns recent or ongoing treatment, specialist consultations and/or medical investigations about matters that a reasonable consumer would have understood to be important to their health • 8.4.3.3 the customer has specialist knowledge – for example, someone in the medical profession or relevant parts of the insurance industry. See Annex, example cases 12 & 13 – misrepresentation of medical information ***** • 8.4.4 Lifestyle information – since lifestyle information is usually more familiar and easier for customers to understand, it follows that customers will need to give a particularly credible and convincing explanation for clearly evidenced misrepresentation not to be classified as deliberate or reckless. See Annex, example cases 14 & 15 – where smoking is not disclosed